

Management of Decompensated Cirrhosis — Clinical One-Pager

Facilitator quick reference · TeachIM (Zhang/Fainstad, Jan 2020), optimized Jun 2026 · CC BY-NC 4.0 · clinician-review before use

Cirrhosis one-liner: cause | compensated vs DECOMPENSATED | MELD 3.0 | prior complications. **Decompensation** = ↑MELD or new complication. Precipitants overlap: infection/SBP, GI bleed, PVT.

Staging & MELD 3.0

Cause: EtOH 60–70%, MASH 10%, HCV/HBV 10%, hemochromatosis, AIH/PSC/PBC.
Compensated median survival >12 y; decompensated much shorter. **MELD 3.0** (Cr/Tbili/INR/Na/albumin/sex) = US allocation (replaced MELD-Na, 2023).

MELD	3-mo mortality
20	3–4%
30	27–32%
40	65–66%

Work-up of Acute Decompensation

- **Diagnostic paracentesis** (SAAG, cell count, culture) — everyone admitted with ascites.
- Evaluate for GI bleeding.
- RUQ US with Doppler (PVT, HCC).
- Recheck MELD components (Cr, Tbili, INR, Na).

Complications & Management

Problem	Management
Ascites / hydrothorax	Na restrict + diuretics (spirono ± furosemide); LVP + albumin if tense
SBP (ANC ≥250)	Cefotaxime/ceftriaxone + albumin 1.5 g/kg d1, 1 g/kg d3
HRS-AKI	Hold diuretics/nephrotox; albumin; terlipressin (1st-line) or octreotide/midodrine + albumin
Hepatic enceph.	Lactulose + rifaximin; treat precipitant; PEG alt
Variceal bleed	Restrictive transfusion (Hgb ~7), octreotide, PPI, ceftriaxone , urgent EGD

2021–2023 Updates

- **MELD 3.0** (2023) replaced MELD-Na (adds albumin + sex).
- **Terlipressin** FDA-approved 2022, first-line for HRS-AKI (type 1 HRS = HRS-AKI).
- **Baveno VII:** carvedilol in compensated cirrhosis + CSPH prevents 1st decompensation.
- NASH → MASH; NAFLD → MASLD (2023).

Pearls

- Missed SBP is a preventable death — tap the belly.
- Albumin in SBP reduces HRS & mortality (esp. Cr >1 or Tbili >4).
- In variceal bleed, ceftriaxone is a survival drug — not optional.
- Restrictive transfusion (Hgb ~7) beats liberal in UGIB.
- Decompensation = put transplant evaluation on the problem list.